Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 04-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Study to Evaluate the Safety and Pharmacokinetics of the Intravenous Fixed-Dose Combination (IV FDC) of Tiragolumab and Atezolizumab in Participants With Locally Advanced, Recurrent or Metastatic Solid Tumors **Short Title/ Acronym:** SKYSCRAPER-11 **Protocol Number:** GO44096 (Other IDs: 2022-001157-23, 2023-508489-14-00) **NCT Number:** NCT05661578 **Posting ID:** 124455473620423458 **Sponsor Name:** Hoffmann-La Roche (Company: Roche) **Trial Status:** ACTIVE_NOT_RECRUITING **Investigational Product:** Tiragolumab and Atezolizumab Intravenous Fixed-Dose Combination (IV FDC) (Drug: atezolizumab | Trade Name: Tecentriq | ATC Code: L01FF05) **Phase of Development:** Phase II **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead / Medical Monitor

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2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To assess the safety, pharmacokinetics (PK), and immunogenicity of tiragolumab and atezolizumab administered as an intravenous fixed-dose combination (IV FDC). * **Secondary Objectives:** To evaluate the preliminary efficacy of the combination, including Overall Survival (OS), Progression-Free Survival (PFS), Objective Response Rate (ORR), and Duration of Response (DOR).

2.2 Background & Rationale Unlike traditional therapies that target cancer cells directly, this dual immunotherapy regimen leverages the immune system. Utilizing Immune Checkpoint Inhibitors, tiragolumab acts as a novel TIGIT inhibitor that helps activate immune cells, while atezolizumab is an established anti-PD-L1 antibody that enhances the body's ability to attack tumor cells. By combining both monoclonal antibodies into a single fixed-dose formulation (in one bottle), the study aims to simplify drug administration, reduce the time patients spend in the clinic, and explore the synergistic immune checkpoint blockade effects in hard-to-treat solid tumors.

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3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Single-arm * **Blinding:** Open-label * **Randomization:** N/A

3.2 Planned Enrollment & Duration * **Target Enrollment:** 60 participants * **Number of Sites:** ~35-61 trial locations globally (including the United States, Europe, and Asia) * **Estimated Study Start:** 04-May-2023 * **Estimated Primary Completion:** 26-Dec-2025

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years, Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1, and life expectancy ≥ 12 weeks. 2. Histologic documentation of locally advanced, recurrent, or metastatic malignancy that is ineligible for definitive local therapy, with measurable disease per RECIST v1.1. 3. Adequate hematologic and end organ function, and recovery (improvement to Grade 1 or better) from all acute toxicities from previous therapy, excluding alopecia. 4. Programmed death-ligand 1 (PD-L1)-selected tumors (requires submittal of archival or fresh tumor tissue for central laboratory evaluation prior to enrollment), as determined by the investigational VENTANA PD-L1 (SP263) immunohistochemistry (IHC) assay. 5. For females of childbearing potential: negative serum pregnancy test within 14 days prior to Day 1, agreement to remain abstinent or use contraception, and agreement to refrain from donating eggs for 5 months post-treatment. For males: agreement to remain abstinent or use condoms, and agreement to refrain from donating sperm for 90 days post-treatment.

4.2 Exclusion Criteria 1. Pregnancy or breastfeeding, or intention to become pregnant. 2. Prior treatment with immune checkpoint inhibitors (participants must be CPI-naive). 3. Any anti-cancer therapy, whether investigational or approved, within 3 weeks prior to initiation of study treatment, or major surgical procedure within 28 days prior to Day 1 of Cycle 1. 4. Significant cardiovascular disease, clinically significant liver disease, or poorly controlled Type 2 diabetes mellitus. 5. History of autoimmune disease, systemic immunosuppressive medications within 2 weeks prior to Day 1, or history of idiopathic pulmonary fibrosis, pneumonitis, or active pneumonitis on screening CT. 6. Severe infections within 4 weeks prior, or recent oral/IV antibiotics within 2 weeks prior to Day 1.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** Fixed-dose combination (FDC) of tiragolumab 600 mg and atezolizumab 1200 mg administered once every 3 weeks (Q3W). * **Route of Administration:** Intravenous (IV) infusion. * **Duration of Treatment:** Day 1 of each 21-day cycle until disease progression, loss of clinical benefit, or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard supportive care for managing cancer-related symptoms and immune-related adverse events (irAEs). * **Prohibited:** Concurrent administration of other anti-cancer/investigational therapies.

Immunomodulators are prohibited within 6 weeks prior to the proposed Day 1 of Cycle 1. Monoclonal antibodies and derived therapies (excluding CPIs) within 5 drug-elimination half-lives (or ~100 days) are prohibited. Cancer vaccines and/or cytokines within 6 weeks or 5 half-lives are also excluded.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Biopsy (PD-L1 test via VENTANA SP263), Baseline Scans (CT/MRI)
Baseline	Day 1 (Cycle 1)	First IV FDC dose, Extensive PK blood sampling, Baseline Safety Labs
Interim Cycles	Every 21 Days ($\pm$ 3)	IV FDC Administration, Safety & PK Labs, Vitals
Tumor Assessment	Every 6 to 9 Weeks	Efficacy Assessment (RECIST v1.1 evaluation)
End of Study	Follow-up	Disease progression confirmation, Survival tracking, Final safety check

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7. Outcome Measures (Endpoints)

7.1 Primary Outcome Measure * **Percentage of Participants With Adverse Events (AEs):** Safety evaluation of the incidence, nature, and severity of Adverse Events, utilizing NCI-CTCAE version 5.0 for AE grading.

7.2 Secondary Outcome Measures * Area Under the Concentration Time Curve (AUC) of Tiragolumab * Area Under the Concentration Time Curve (AUC) of Atezolizumab * Maximum Serum Concentration (Cmax) of Tiragolumab * Additional Exploratory/Efficacy measures: Objective Response Rate (ORR), Progression-Free Survival (PFS), Overall Survival (OS), and Duration of Response (DOR).

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8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Continuous monitoring at every onsite visit. Specific focus on identifying immune-related adverse events (irAEs) such as rash, arthralgia, and infusion-related reactions. Patients with history of immune-mediated Grade 4 AEs or unresolved Grade 3 AEs from prior cancer immunotherapy are excluded.
- **Serious Adverse Events (SAEs):** Routine reporting to sponsor and regulatory bodies within 24 hours of investigator awareness.
- **Data Monitoring Committee (DMC):** Internal safety review committee established to monitor the safety of the novel IV fixed-dose formulation combination.

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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Safety Set (all patients receiving ≥ 1 dose of study drug); PK-Evaluable Population; Efficacy Evaluable Set (ITT).
- **Sample Size Justification:** $N=60$. As a Phase II safety and PK study, this sample size is adequate to estimate pharmacokinetic parameters and capture the incidence rate of common AEs and objective responses.
- **Handling of Missing Data:** Standard survival analysis methods (e.g., Kaplan-Meier estimates and right-censoring for patients lost to follow-up) for time-to-event secondary endpoints.

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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
- **Monitoring Plan:** Routine onsite and centralized monitoring of electronic Case Report Forms (eCRFs) by Hoffmann-La Roche.
- **Audit Trail:** eCRF locking, source data verification, and secure biospecimen tracking for PK/PD-L1 assays.

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11. Study Identifiers & Governance

- **Primary ID**: GO44096
- **Posting ID**: 124455473620423458
- **Registry Number:** NCT05661578
- **Sponsor/Lead Organization:** Hoffmann-La Roche (Company: Roche)
- **Trial Status:** ACTIVE_NOT_RECRUITING
- **Study Title:** A Phase II, single-arm, open-label study evaluating the safety and pharmacokinetics of the intravenous fixed-dose combination (IV FDC) of tiragolumab and atezolizumab in participants with locally advanced, recurrent, or metastatic solid tumors

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12. Clinical Framework (Solid Tumor Specific)

- **Disease Area:** PD-L1-selected Solid Tumors
- **Primary Condition:** Locally Advanced, Recurrent or Metastatic Solid Tumors
- **Diagnosis Threshold:** Histological documentation of malignancy + PD-L1 positive tissue status
- **Medical Methodology:** Immune Checkpoint Inhibition (TIGIT + PD-L1 co-inhibition)
- **Optimization Target:** Adequate pharmacokinetics, safety profile (Maximum Tolerated Dose equivalent), and objective tumor shrinkage

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13. Study Procedures & Methodology

- **Management Pathway:** Intravenous (IV) Infusion Oncology Clinical Pathway
- **Education Protocol (HCP):** Site initiation and training on IV FDC administration protocols and immune-related AE (irAE) recognition.
- **Education Protocol (Patient):** Education regarding potential immune-mediated side effects (e.g., severe rash, breathing difficulty, liver/GI issues) and mandatory reporting expectations.
- **Quality Audit Mechanism:** Continuous PK/PD sample integrity audits and clinical site monitoring for proper RECIST v1.1 application.

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14. Observation & Follow-Up Schedule

- **Total Duration:** Until disease progression or unacceptable toxicity (estimated up to 2–3 years for survival follow-up).
- **Onsite Visit Cadence:** Every 3 weeks (Day 1 of each 21-day cycle).
- **Remote Monitoring Frequency:** Between treatment cycles as necessary for adverse event follow-up.
- **Checklist Review Interval:** Tumor assessments (imaging) every 6 to 9 weeks.

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15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- |
| **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] |** ___ **|** ***[DD-MMM-YYYY]* | | *Lead Statistician* | *[Name]* |** _____ | [DD-MMM-YYYY] |